Benevolent House

T3C Service Delivery Addendum to the Policy and Operations Manual

Purpose: This addendum sets the policies and procedures Benevolent House follows to deliver services under the Texas Child-Centered Care (T3C) System.

Service Packages: T3C Basic Foster Family Home Support Services; Mental & Behavioral Health Support Services; Kinship Caregiver Support Services Add-On

Version 3, revised October 23, 2026. Incorporates Version 2 (DFPS enhancement response, October 15, 2026).

T3C-1 Scope and Authority

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To state which services this addendum covers and how it relates to the Benevolent House Policy and Operations Manual.

POLICY

Benevolent House delivers T3C services according to the T3C System Blueprint published by the Texas Department of Family and Protective Services, the Minimum Standards for Child-Placing Agencies, and its contracts with DFPS and each Single Source Continuum Contractor (SSCC). This addendum applies to every child, youth, and young adult served under a T3C Service Package or Add-On Service for which Benevolent House holds a Credential.

PROCEDURE

1. The Policy and Operations Manual remains in effect. This addendum adds the requirements that are specific to T3C.
2. Where this addendum and the manual differ for a child served under T3C, this addendum controls.
3. The Executive Director reviews this addendum each time DFPS publishes a new edition of the T3C System Blueprint and brings needed changes to the Board of Directors.
4. Staff and caregivers are trained on this addendum before Benevolent House begins operating under an Active Full Credential and whenever it changes.

T3C-2 Treatment Model Integration

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026 **Revised:** October 23, 2026

PURPOSE

To make sure Trauma Systems Therapy for Foster Care (TST-FC) is practiced throughout Benevolent House and is the basis for policy, procedure, and practice in each Service Package and Add-On Service.

POLICY

TST-FC is the Treatment Model for all children served by Benevolent House under T3C. Every staff member, caregiver, and contracted provider who serves these children is trained in the model, practices it, and stays current through ongoing training: four one-hour TST-FC sessions a year, one each calendar quarter, delivered live, by video conference, through the Benevolent House online training portal, or one-to-one at a home visit, as set out in the TST-FC Ongoing Training Curriculum, with the sessions required of each contracted role stated in the T3C Training Curriculum and Plan. Each child's Service Plan is written in the terms of the model. The model is applied to the custom needs of each Service Package and Add-On Service as set out in Section G.

PROCEDURE

A. Admission and matching

1. Admissions staff review the Application for Placement, the child's trauma history and known triggers, and the CANS 3.0 Assessment when one has been completed.
2. Staff select a caregiver who is credentialed for the child's Service Package, has completed TST-FC training, and can provide the level of predictability and supervision the child needs.
3. Before or at placement, the case manager tells the caregiver the child's known triggers, early warning signs, and what has helped the child calm in the past.

B. Service planning

1. Each Service Plan includes, in TST-FC terms: the child's priority problem; known triggers; signals of safety; regulation strategies for each of the four stages of behavior; and goals for both the child and the caregiving environment.
2. The child takes part in service planning when appropriate for the child's age and development, and the child's own words are used for triggers and calming strategies where possible.
3. Results of the most recent CANS 3.0 Assessment are used to set and adjust the type, frequency, and duration of services.
4. Each Service Plan review documents progress toward each goal and any change to the Managing Emotions Guide.

C. Case management and caregiver coaching

1. At each monthly visit the case manager asks about episodes since the last visit, uses the moment-by-moment assessment to look at what happened before, during, and after, and coaches the caregiver on the next step.
2. The case manager records the coaching provided and the caregiver's use of the model in the visit note.
3. The quarterly foster home evaluation includes a review of the caregiver's understanding and use of TST-FC.

D. Therapeutic and clinical services

1. Therapy is arranged through STAR Health at the frequency the Service Planning team determines, and the Service Plan states the assessed need and frequency.
2. The case manager shares the child's priority problem and Managing Emotions Guide with the child's therapist and invites the therapist to each Service Plan meeting.
3. For the Mental & Behavioral Health Support Services Package, the Treatment Director and the Licensed Therapist review treatment and service planning as described in Section G.

E. Behavior support and discipline

1. Caregivers use predictability, pre-teaching, staying neutral, time-in, and restorative discipline.
2. Punishment that shames, isolates, or repeats a child's experience of harm is not used.
3. After any serious incident or emergency behavior intervention, the case manager completes a moment-by-moment review with the caregiver and updates the Managing Emotions Guide when the review shows a new trigger or a strategy that did not work.

F. Recreation, Normalcy, and family connection

1. Caregivers plan Normalcy activities that build regulation, resilience, and connection, and plan ahead for times that are hard for the child.
2. Caregivers and case managers support contact with parents, siblings, relatives, and other supportive persons when it is safe and approved by the DFPS or SSCC caseworker.

G. Programming by Service Package

Each Service Package and Add-On Service serves a population with custom needs. The table states those needs, how the model is designed to meet them, and the programming specific to the package. The Treatment Model Narrative, Section 4, describes each in full.

Service Package	Custom needs, how the model is designed for them, and programming specific to the package
T3C Basic Foster Family Home Support Services	**Custom needs.** Children removed from home because of abuse or neglect whose needs are relational and developmental rather than clinical: a predictable home, adults whose reactions can be anticipated, help with ordinary moments their history has made unsafe, continuity in school and community, and protection of ties to family and culture. **How the model is designed for them.** Predictability and routines are the first intervention. The Managing Emotions Guide is the working document. Pre-teaching and signals of safety are built into everyday parenting. The case manager coaches the caregiver at each monthly visit with the Moment-by-Moment Assessment. TST-FC is applied in everyday parenting: routines, transitions, pre-teaching, and signals of safety. A Managing Emotions Guide is built with each child old enough to take part. Service Plan review at least every six months. CANS 3.0 Assessment annually for children over age 3. The Program Director reviews goals and services with each six-month review. **Protection from re-traumatization.** Restorative discipline only; no shame, isolation, withholding of food or family contact, or consequence that repeats a child's experience of harm. No processing of an episode while the child is in a survival state. Known triggers shared with the caregiver before or at placement. Family and sibling connections protected. When a move is unavoidable, the Managing Emotions Guide and strategies go with the child.
Mental & Behavioral Health Support Services	**Custom needs.** Children who have, or are being evaluated for, an emotional, conduct, or behavioral disorder: more frequent, intense, and dangerous episodes; therapy and often medication that must be coordinated with the home; higher risk of crisis, serious incident, hospitalization, and disruption; caregivers at higher risk of burnout. They need the home and the clinic working from one plan, adults who stay regulated through a long episode, and a path back to the Basic package in the same home. **How the model is designed for them.** All Basic programming, plus a written safety and crisis plan tied to the four stages, one plan across home and clinic through the Treatment Director and Licensed Therapist, review every 90 days, crisis response that uses the child's own guide, more coaching for the caregiver, and step-down to Basic in the same home. All Basic programming applies. A written safety and crisis plan is tied to the child's four stages of behavior and is shared with the caregiver, therapist, and on-call staff. Case manager level or above staff respond to any crisis 24 hours a day, 7 days a week. Caregivers take part in therapy with the child as needed, attend multiple meetings, and respond immediately to the child's mental and behavioral health needs. The Licensed Therapist provides oversight and input on treatment and service planning, including the frequency of therapy. Service Plan review and CANS 3.0 Assessment at least every 90 days. The Program Director and Treatment Director provide written continued stay confirmation with each review. An Aftercare Services plan is prepared at discharge, with follow-up for six months. **Protection from re-traumatization.** Everything in Basic, plus: emergency behavior intervention is a last resort within the caregiver's Minimum Standards training and is followed every time by a Moment-by-Moment review and an update to the guide and crisis plan; in a survival moment the other children are moved, not the child in crisis; psychotropic medication is monitored with the medical consenter and the child's input; hospitalization and placement change are planned whenever possible and the child's guide travels with the child; caregiver exhaustion is treated as a clinical risk to the child and met with paid Intermittent Alternate Care and added support.

Service Package	Custom needs, how the model is designed for them, and programming specific to the package
Kinship Caregiver Support Services Add-On	**Custom needs.** Relatives and fictive kin who did not plan to foster and often took the child on short notice; the child's trauma is also the family's trauma. They need the model in plain language around their own family history, practical help with verification, benefits, and deadlines, support on evenings and weekends, and help keeping the child's connection to parents and siblings safe and steady. **How the model is designed for them.** The model is taught one-to-one, including each quarterly session offered at a home visit using an episode from the caregiver's own home; family history is part of the assessment; Kinship Caregiver Home Support staff carry the practical load; family connection is treated as the goal; aftercare continues after permanency. Provided together with the child's primary Service Package. TST-FC is taught to each kinship caregiver individually, with attention to family history, grief, and the caregiver's relationship with the child's parents. Kinship Caregiver Home Support staff help the caregiver through verification, benefits applications, and the first months of placement, including evenings and weekends. An Aftercare Services plan is prepared when the child reaches permanency with the kinship caregiver. **Protection from re-traumatization.** Everything in the child's primary package, plus: the child is kept out of adult conflict about the parents; the caregiver is not left to learn the model alone, with the first sessions and coaching offered in the home; grief and loss are named for the caregiver and the child; verification and benefits are handled so that financial strain does not become the reason the child moves again.

H. Training and oversight

1. Training requirements are set out in the Benevolent House T3C Training Curriculum and Plan and the TST-FC Ongoing Training Curriculum. Every staff member and caregiver completes the eight-hour TST-FC training before serving a child under T3C and four one-hour TST-FC sessions each year, one each quarter. Each session ends with the participant applying the skill to a child in their care, and the case manager reviews the result at the next home visit.
2. The Treatment Director leads fidelity to the model. The Program Director makes sure it is carried out in daily operations. The Case Manager Supervisor confirms it is reflected in Service Plans, visit notes, and quarterly evaluations.
3. Use of the model is reviewed each quarter through the Continuous Quality Improvement process.

T3C-3 Child, Youth, and Young Adult Education and Awareness of the Treatment Model

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026 **Revised:** October 23, 2026

PURPOSE

To make sure each child, youth, and young adult served under T3C is aware of the Treatment Model being used to support them, in a way that fits their age and development.

POLICY

Children are partners in their own care. Benevolent House explains TST-FC to each child in words the child can use, builds the child's plan for managing emotions with the child, and returns to it throughout the placement.

PROCEDURE

A. Methods by age and development

Age	Method
Birth through age 4	Awareness is built through experience. The caregiver provides predictable routines, names feelings out loud, and uses signals of safety. The case manager documents the caregiver's practices in the Service Plan.

Age	Method
Ages 5 through 11	The case manager and caregiver explain, in simple words, that big feelings are the body's alarm and that the adults in the home have a plan to help. The caregiver builds a Managing Emotions Guide with the child using a feelings scale and the child's own choices of what helps. The child receives the guide titled What Helps Me Feel Safe (Attachment A).
Ages 12 through 17	The case manager explains how stress and trauma affect the brain and body, the four stages of behavior, and how the Service Plan uses them. The youth builds the Managing Emotions Guide with the caregiver, takes part in Service Plan meetings, and receives Attachment A.
Young adults	The case manager reviews the model with the young adult as a set of skills for adult life, and the young adult leads the update of their own Managing Emotions Guide and Service Plan goals.

B. Steps and timeframes

1. **At admission.** The case manager tells the child, in age-appropriate words, how Benevolent House caregivers help children with big feelings.
2. **By the initial Service Plan.** The caregiver and child complete the Managing Emotions Guide. The case manager reviews Attachment A with the child.
3. **At each monthly visit.** The case manager checks in with the child using the language of the model and asks what is helping and what is not.
4. **At each Service Plan review.** The child's Managing Emotions Guide is updated with the child, and the child's view of their progress is recorded.
5. **When the child changes homes or Service Package.** The Managing Emotions Guide moves with the child and is reviewed with the new caregiver.

C. Documentation

1. The Service Plan records the date the model was explained to the child and the date the Managing Emotions Guide was completed or updated.
2. Monthly visit notes record the check-in with the child.
3. When a child is too young or is unable to take part, the Service Plan states why and describes the caregiver practices used in place of direct education.
4. The Case Manager Supervisor reviews this documentation at each Service Plan review, and completion is reported through the Continuous Quality Improvement process.

D. Integration into daily care

Awareness is kept alive in the ordinary day, not only at meetings. Caregivers make the model visible to the child in the following ways, and the case manager checks for them at each monthly visit.

1. The caregiver uses the child's own words from the Managing Emotions Guide: the child's names for the stages (for example, calm, getting upset, alarm is on, coming back) and the calming strategies the child chose.
2. The caregiver names feelings out loud, for the child and for themselves, so the child learns that feelings are noticed and survivable.
3. The caregiver pre-teaches what is coming next, at transitions, before visits, and before changes in plan, so the child is not surprised.
4. The caregiver keeps the routines the child helped set, and tells the child ahead of time when a routine must change.
5. The caregiver offers a do-over instead of a lecture, and repair instead of punishment, so the child experiences the model as safety rather than as rules.
6. For children under five, the caregiver provides these practices without explanation; awareness is built through experience.

7. The child's guide, What Helps Me Feel Safe, is kept where the child can reach it, and the Managing Emotions Guide is posted or kept in a place the child chose.
8. Youth and young adults are told, in plain words, that the caregiver is using TST-FC and why, and are invited to say what is working and what is not at any time, not only at the monthly visit.

T3C-4 Admission Guidelines

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To set the guidelines Benevolent House uses to decide whether a child is admitted under each T3C Service Package and Add-On Service.

POLICY

Benevolent House admits a child under a T3C Service Package when the package fits the child's needs and strengths and a credentialed caregiver is available who can meet them. These guidelines are applied together with the admission requirements of Minimum Standards and Section VI of the Policy and Operations Manual, Admissions and Placement.

PROCEDURE

A. Guidelines that apply to every admission

1. The placement type and Service Package align with the child's needs and strengths, as shown by the CANS 3.0 Assessment when one was completed before the referral, the Application for Placement, and the knowledge and professional judgment of the child's Service Planning team.
2. A pre-placement visit has taken place when applicable and appropriate, and it was successful.
3. Admissions staff have reviewed the child's information and determined that the child's needs align with the services Benevolent House offers and with the selected caregivers.
4. Benevolent House and the selected foster family home hold the Credential for the Service Package.

B. Guidelines by Service Package

Service Package	The child is admitted under this package when
T3C Basic Foster Family Home Support Services	The child's needs can be met in a trauma-informed foster family home with community-based services, routine therapy as determined by the Service Planning team, and coordination of medical, dental, educational, and behavioral health care. The child does not require the added clinical oversight of a specialized Service Package.
Mental & Behavioral Health Support Services	The child has, or is being evaluated for, a DSM-5 diagnosis of an emotional, conduct, or behavioral disorder, and routine clinical intervention such as therapy, education, or medication is needed to support and manage day-to-day activities. The admission assessment includes the psychiatric evaluation, psychological evaluation, or psychosocial assessment required by Minimum Standards for a child receiving treatment services for an emotional disorder. The Program Director has consulted the Treatment Director about the child's clinical needs. The selected home is verified to provide treatment services for emotional disorders and is credentialed for both this package and the Basic package.
Kinship Caregiver Support Services Add-On	The child is placed with a relative or fictive kin caregiver whose home is verified by Benevolent House as a foster family home. The child is receiving a primary T3C Service Package from Benevolent House. The Add-On Service is always delivered together with the primary package.

C. Steps

1. Admissions and placement staff are on call 24 hours a day, 365 days a year, to screen referrals and admit children.

2. Admissions staff record every referral in the T3C Referral, Admission, and Discharge Tracker on the day it is received, with the referral source and the Service Package requested.
3. Admissions staff review the referral against the guidelines in A and B and identify a caregiver as described in Policy T3C-2, Section A.
4. When a child may fit more than one Service Package, the primary Service Package is determined by agreement between Benevolent House and the SSCC or DFPS.
5. Admissions staff give the SSCC or DFPS an answer on every referral. When a child is not admitted, the reason is stated in the response and recorded in the tracker.
6. The Program Director approves each admission and consults the Treatment Director on admissions under the Mental & Behavioral Health Support Services Package. For a child on the Program Director's own caseload, the Executive Director approves.
7. The admission decision, the Service Package, and the basis for the decision are documented in the child's case record.

T3C-5 CANS 3.0 Assessment, Service Planning, and Arranging Services

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026 **Revised:** October 15, 2026

PURPOSE

To set the day-to-day procedures for assessment, service planning, therapy and other services, and care and supervision plans for children served under T3C.

POLICY

Each child has a customized Service Plan that is informed by the child, built with the Service Planning team, guided by the most recent CANS 3.0 Assessment, and written in the terms of the Treatment Model. Benevolent House arranges every service the plan calls for and documents the result.

PROCEDURE

A. Schedule

Requirement	Basic	Mental & Behavioral Health
CANS 3.0 Assessment, children over age 3, youth, and young adults	At least once every 12 months.	At least once every 90 days.
Service Plan review	At least once every six months, and more often when the child's needs or permanency plan call for it.	At least once every 90 days, and more often when the child's needs or permanency plan call for it.
Therapy	Individual and family therapy at the frequency set by the Service Planning team.	Individual, family, and group therapy at the frequency set by the Service Planning team and the Licensed Therapist.
Crisis response	On-call staff are available to caregivers 24 hours a day, 7 days a week.	Case manager level or above staff respond to any crisis in person, by phone, or by video conference 24 hours a day, 7 days a week.

A child receiving the Kinship Caregiver Support Services Add-On follows the schedule of the child's primary Service Package.

B. CANS 3.0 Assessment

1. The case manager tracks the due date of each child's CANS 3.0 Assessment and makes sure it is completed by a certified CANS assessor within the timeframe in Section A.
2. The case manager obtains the results and files them in the child's case record.

3. At each Service Plan review, the Service Planning team reviews the most recent results and uses them to set or adjust the type, frequency, and duration of services. The Service Plan review states which results were considered and what changed because of them.
4. The Case Manager Supervisor confirms at each review that the assessment is current.

C. Service Plan

1. The Service Plan includes customized goals and the planned services and supports that will help the child reach each goal, along with the Treatment Model content required by Policy T3C-2, Section B.
2. The child, when appropriate, and each youth and young adult help shape the plan. Parents, relatives, and supportive persons are included as described in Policy T3C-8.
3. Each Service Plan review documents the progress made toward each goal.

D. Arranging therapy and other services

1. Therapy and other Medicaid-covered services are arranged through STAR Health with providers who are credentialed and contracted with STAR Health. The case manager sends authorization requests to STAR Health as needed.
2. The Service Plan includes a written justification of the child's assessed need for therapy, including the frequency.
3. For the Mental & Behavioral Health Support Services Package, therapy is provided by a Licensed Therapist with experience treating children with emotional, behavioral, and conduct disorders, unless the Service Planning team determines that a different type of therapist is needed. Foster family home caregivers participate in therapy with the child as needed. Caregivers must have the ability to attend multiple meetings and to respond immediately based on the child's mental and behavioral health needs. The case manager confirms this ability when the home is credentialed and re-credentialed for this Service Package under Policy T3C-10 and Attachment D, and reviews it at each 90-day Service Plan review.
4. In addition to holding the Credential for the Mental & Behavioral Health Support Services Package, Benevolent House and each foster family home that serves a child under that package also hold the Credential for the T3C Basic Foster Family Home Support Services Package. This allows the child to transition to, and receive, the T3C Basic Foster Family Home Support Services Package in the same foster family home when the SSCC or DFPS, in collaboration with Benevolent House and as informed by the Quality Assurance and Continued Stay Guidelines in Policy T3C-6, determines that the child has successfully completed, and no longer requires, the therapeutic and recovery services of the Mental & Behavioral Health Support Services Package.
5. The case manager and caregiver coordinate care with the child's medical consenter and take part in STAR Health Service Coordination when the child is eligible.
6. The case manager acts as the child's education liaison: takes part in school enrollment, requests educational testing when needed, takes part in Admission, Review, and Dismissal and Section 504 meetings as needed, and confirms that accommodations and supports are in place.
7. Caregivers provide transportation, logistical support, and record keeping for services as required by court orders and the Service Plan.

E. Documenting services and access to services

1. In collaboration with the medical consenter, the case manager documents in the case record all services the child receives through STAR Health, HHSC Behavioral Health Services, Early Childhood Intervention, the school, and any other county, community, or state agency.
2. When a service called for in the Service Plan is not readily available, or the child is found ineligible, the case manager documents the date of the request, application, or referral; the type of service requested; the status of the request; the reason given for any denial; and the status of any appeal.
3. The case manager notifies the SSCC or DFPS caseworker within 3 business days of any difficulty obtaining a service or any denial of a service referral.
4. The case manager seeks community resources for any needed service that STAR Health does not cover.

5. When a service called for in the Service Plan is consistent with the child's Service Package and is not available through STAR Health or community resources, the case manager brings the need to the Program Director, and Benevolent House makes sure the service is delivered and covers its cost.

F. Care and supervision plans

1. The Service Plan states the supervision the child requires and any special care instructions.
2. Each child in the Mental & Behavioral Health Support Services Package has a written safety and crisis plan tied to the four stages of behavior. The plan is shared with the caregiver, the therapist, and on-call staff, and is reviewed at each 90-day review and after any serious incident.
3. Benevolent House foster family homes care for no more than six children. If HHSC Child Care Regulation grants a variance that allows a home to care for seven or more children, awake night supervision is provided in that home.
4. Caregivers support Normalcy activities that fit the child's age and Service Plan, including clothing, hygiene products, hair care, birthdays, holidays, and graduations.

T3C-6 Quality Assurance and Continued Stay Guidelines

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To make sure each child continues to receive the Service Package that fits the child's needs, and that every required review, written confirmation, and notification is completed.

POLICY

Benevolent House reviews each child's services on a set schedule. At each review, the responsible directors confirm in writing that the child continues to meet the criteria for the Service Package and is benefitting from the Treatment Model. A child whose needs have changed is moved to the package that fits.

PROCEDURE

A. Continued stay guidelines

A child continues in a Service Package when all of the following are true.

1. Services are being reviewed and adjusted based on the most recent CANS 3.0 Assessment and the Service Plan.
2. The primary reason the child met the Admission Guidelines continues to require ongoing services, or that reason has been replaced by other service needs that fit the Service Package and meet the Admission Guidelines.
3. The services continue to support the child's need for safety, improved well-being, and permanency, in accordance with the child and family Service Plans.
4. A less restrictive placement type is not appropriate to meet the child's needs.
5. For the Mental & Behavioral Health Support Services Package, the child's needs continue to call for a level of support that the Basic package does not provide.
6. Benevolent House and the foster family home continue to hold the Credential for the Service Package.

B. Review and written confirmation

Service Package	When	Who reviews and signs	What the written confirmation states
T3C Basic Foster Family Home Support Services	With each six-month Service Plan review	Program Director	The child continues to meet the criteria for the package and is benefitting from the Treatment Model.
Mental & Behavioral Health Support Services	With each 90-day Service Plan review	Program Director and Treatment Director	The child continues to meet the criteria for the package and is benefitting from the Treatment Model, and the Basic package is not appropriate to meet the child's needs.

1. The case manager gives the reviewing directors the most recent CANS 3.0 Assessment results, the Service Plan review, and the progress recorded for each goal.
2. The directors review the child's goals and services to make sure they align with the child's strengths, needs, and permanency plan, and apply the guidelines in Section A.
3. The directors complete and sign the Continued Stay Written Confirmation (Attachment B). For a child on the Program Director's own caseload, the Executive Director signs in the Program Director's place, as stated in Policy T3C-S1.
4. The signed confirmation is filed in the child's case record.
5. The case manager sends a copy to the SSCC or DFPS caseworker within 15 business days of the review and confirmation, and records the date sent on the form.

C. When a child no longer meets the guidelines

1. When the review shows that a child in the Mental & Behavioral Health Support Services Package has completed, and no longer requires, the therapeutic services of that package, the Program Director notifies the SSCC or DFPS caseworker. When the SSCC or DFPS agrees, the child moves to the Basic package and remains in the same foster family home.
2. When the review shows that a child needs a Service Package that Benevolent House does not hold, the Program Director notifies the SSCC or DFPS caseworker and takes part in planning the transition. The child's Managing Emotions Guide and records go with the child.
3. The change is recorded in the T3C Referral, Admission, and Discharge Tracker with the date and the reason.

D. Quality assurance

1. The Case Manager Supervisor tracks the due date of every Service Plan review, CANS 3.0 Assessment, and written confirmation.
2. On-time completion of reviews and confirmations is reviewed each quarter through the Continuous Quality Improvement process.

T3C-7 Anticipated Length of Service

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To state the length of service Benevolent House anticipates for each Service Package and how actual length of service is tracked.

POLICY

Length of service is individualized. It is based on the Treatment Model, the Admission Guidelines, the Continued Stay Guidelines, the child's CANS 3.0 Assessment, and the child's progress under the Service Plan. No child is discharged or moved to another Service Package because an anticipated length of service has been reached. That decision is made only through the Continued Stay Guidelines.

PROCEDURE

Service Package	Anticipated length of service	Basis
T3C Basic Foster Family Home Support Services	12 to 18 months	Services continue until the child reaches permanency through reunification, placement with a relative, adoption, or transition to adult living.
Mental & Behavioral Health Support Services	6 to 12 months	Reviewed every 90 days. When the child no longer requires the therapeutic services of this package, the child moves to the Basic package in the same foster family home.

Service Package	Anticipated length of service	Basis
Kinship Caregiver Support Services Add-On	6 to 12 months after the home is verified, followed by 6 months of Aftercare Services	Services continue while the child is placed in the verified kinship foster family home and end when the child reaches permanency.

1. The anticipated length of service is discussed with the Service Planning team at the initial Service Plan and at each review.
2. The T3C Referral, Admission, and Discharge Tracker records the start date, end date, and end reason for each child under each Service Package.
3. When a child changes Service Package, the current line is closed with the reason "Service Package change" and a new line is started, so that length of service is measured separately for each package.
4. Average length of service by Service Package, from admission to discharge, is reviewed each quarter through the Continuous Quality Improvement process. The anticipated lengths in this policy are reviewed each year against that data.

T3C-8 Engagement of the Child, Family, and Support Network

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To set the approach Benevolent House uses to reach out to, engage, and include the child and the people who matter to the child, and to state where that inclusion is documented.

POLICY

To the extent that it is safe and appropriate, and in collaboration with the SSCC or DFPS caseworker, Benevolent House and its caregivers reach out to, engage, and collaborate with the child, the child's parents, siblings and other relatives, potential kinship and fictive kin caregivers, adoptive caregivers, and other supportive persons. They are included in care coordination and service planning for the whole placement and, when appropriate, during Aftercare Services.

PROCEDURE

A. Identifying the child's family and supportive persons

1. At the initial Service Plan and at each Service Plan review, the case manager talks with the child about the people who are important to the child: parents, siblings, relatives, former caregivers, teachers, coaches, mentors, and friends.
2. The case manager records the names, relationships, and any contact information in the Family and Support Network section of the Service Plan.
3. The case manager shares each newly identified family member or supportive person with the SSCC or DFPS caseworker promptly, and no later than the next monthly visit, and records the date shared.

B. Including each person

1. The case manager confirms with the SSCC or DFPS caseworker which persons may safely be contacted and included, and follows court orders and the visitation plan.
2. Approved persons are invited to Service Plan meetings in the way that works for them: in person, by phone, or by video conference. Meetings are scheduled with their availability in mind.
3. A person who cannot attend is asked for input beforehand and receives a summary afterward when the caseworker approves.
4. Caregivers support contact, including sibling contact, and share information about the child's daily life, school, and health with parents and relatives as approved by the caseworker.
5. The child takes part in the child's own Service Plan meetings as appropriate to age and development.

C. Where inclusion is documented

Record	What is documented
Service Plan, Family and Support Network section	Each person identified, the relationship, the date shared with the caseworker, and whether contact is approved.
Service Plan and Service Plan review, participants section	Who was invited, how each person was invited, who took part and how, and the input of anyone who did not attend.
Monthly visit notes	Outreach and contacts with family and supportive persons since the last visit, and the caregiver's support of those contacts.
Aftercare contact log	Family and supportive persons involved during the Aftercare Services period.

When a person is not included, the Service Plan states the reason, such as a court order, a safety concern identified by the caseworker, or the person's own choice.

T3C-9 Aftercare Services

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To set how Benevolent House plans and provides Aftercare Services for the Mental & Behavioral Health Support Services Package and the Kinship Caregiver Support Services Add-On.

POLICY

Every child who leaves the Mental & Behavioral Health Support Services Package, whether the discharge is successful or not, leaves with an Aftercare Services plan and receives follow-up for six months. Every kinship caregiver receiving the Add-On Service receives an Aftercare Services plan and six months of follow-up when the child reaches permanency with that caregiver. The T3C Basic package does not include Aftercare Services. Discharge planning for those children follows the Service Plan and the Policy and Operations Manual.

PROCEDURE

A. Requirements by Service Package

	Mental & Behavioral Health	Kinship Caregiver Support Add-On
When the plan is prepared	Before every discharge from the package, successful or unsuccessful. Planning begins at admission and is updated at each 90-day review.	When the child reaches permanency with the kinship caregiver through adoption or Permanent Managing Conservatorship. Also when the child is placed temporarily elsewhere and the caseworker intends for the child to return to the kinship home.
Who prepares it	The Aftercare Case Manager with the child, the caregiver, the Treatment Director, and the next caregiver or family.	The Aftercare Kinship Support staff member together with the kinship caregiver.
What the plan includes	Name and contact information for the STAR Health Service Coordinator, when one is assigned, and the Aftercare Case Manager. Referrals for continued services. The child's Education Portfolio. Initial appointments already set when a change of provider is needed. The child's Managing Emotions Guide and the strategies that have worked. The dates and times of follow-up contacts.	Name and contact information for the Aftercare Kinship Support staff member. Referrals for benefits, support, and continued services in the home. Information on kinship support groups. The dates and times of follow-up contacts.

	Mental & Behavioral Health	Kinship Caregiver Support Add-On
Follow-up contacts	Calls or meetings twice a month for 6 consecutive months.	Calls or meetings weekly for the first month, then twice a month, for 6 months following discharge.

B. Steps

1. The plan is written on the Aftercare Services Plan (Attachment C) or within the child's Service Plan. A copy is given to the child when appropriate, the caregiver or family receiving the child, and the SSCC or DFPS caseworker at discharge.
2. The Aftercare Case Manager or Aftercare Kinship Support staff member makes each scheduled contact by phone, by video conference, or in person.
3. Additional meetings, staffings, and referrals for new services are provided during the six months when the child or family needs them.
4. When a contact cannot be completed, the staff member documents the attempt and tries again.
5. Benevolent House provides, or refers each kinship caregiver to, a kinship support group as part of aftercare.
6. Family and supportive persons are included in aftercare as described in Policy T3C-8.
7. As the child's situation calls for, aftercare planning also includes a CANS 3.0 Assessment reviewed with the Service Planning team before the transition, help with school enrollment, steps to prevent a lapse in therapy or medication when providers change, and encouragement to enroll with the Post-Adoption Support Services provider when a child is adopted.

C. Documentation

1. Every contact, attempted contact, referral, and other case management support during the aftercare period is recorded in the aftercare contact log in the child's case record.
2. A copy of that month's documentation is sent to the SSCC or DFPS caseworker at the end of each month of the aftercare period.
3. Outcomes during aftercare, including placement stability and continued services, are recorded and reviewed through the Continuous Quality Improvement process.

T3C-10 Credentialing and Re-Credentialing Foster Family Homes

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026 **Revised:** October 23, 2026

PURPOSE

To set the process and requirements Benevolent House uses to assess and credential a foster family home for a T3C Service Package or Add-On Service, and the process and frequency for re-credentialing.

POLICY

A child is served under a T3C Service Package only in a foster family home that Benevolent House has credentialed for that package. Credentialing is in addition to verification under Minimum Standards. Every credentialed home is re-assessed and re-credentialed every 12 months.

PROCEDURE

A. Requirements

Credential	Requirements for the foster family home
T3C Basic Foster Family Home Support Services	Current verification as a Benevolent House foster family home, in good standing with Minimum Standards. Each caregiver has completed the eight-hour TST-FC training, passed the Final Knowledge Check, and is current on the quarterly TST-FC sessions set out in the TST-FC Ongoing Training Curriculum. Each caregiver has completed the Universal Human Trafficking Prevention Training. Each caregiver has completed the T3C Policy and Procedure Training. All pre-service or annual training, background checks, and home documents are current. The most recent quarterly foster home evaluation shows the caregiver understands and uses the Treatment Model.
Mental & Behavioral Health Support Services	All requirements for the Basic credential. The home holds both credentials so that a child can move to the Basic package without changing homes. The home is verified to provide treatment services for emotional disorders. Each caregiver has completed the emergency behavior intervention training required by Minimum Standards for treatment services, suicide prevention training, and psychotropic medication training. The caregivers are able to take part in therapy with the child, attend multiple meetings, and respond immediately to the child's mental and behavioral health needs.
Kinship Caregiver Support Services Add-On	The caregiver is a relative or fictive kin of the child, and the home is verified by Benevolent House as a foster family home. The home holds the credential for the child's primary Service Package. Kinship Caregiver Home Support staff help the caregiver complete the training on a schedule that fits the family. A kinship support plan is in place that lists the benefits, training, and supports the caregiver needs.

B. Assessment and credentialing process

1. The case manager reviews the foster home record against the requirements in Section A, visits the home, and talks with the caregivers about the Service Package and what it asks of them.
2. The case manager completes the Foster Family Home T3C Credential Record (Attachment D) and makes a recommendation.
3. The Case Manager Supervisor reviews the record. The Program Director approves each credential.
4. The signed Credential Record is filed in the foster home record. The home's credentials and the date each is due for renewal are entered in the compliance tracker.
5. A home that does not yet meet a requirement receives a written list of what remains, and the case manager helps the caregivers complete it.
6. For homes that are verified before Benevolent House begins operating under the Active Full Credential, the assessment is completed before the first child is served in that home under a T3C Service Package.

C. Re-assessment and re-credentialing

1. **Frequency.** Each credentialed home is re-assessed and re-credentialed every 12 months, in the month of the home's original credential.
2. **Process.** The case manager repeats the review in Section B using the last four quarterly foster home evaluations, the annual training record including completion of the four quarterly TST-FC sessions, and outcomes for the children served in the home. A new Credential Record is completed and approved.
3. **Changes during the year.** When the household moves, a caregiver joins or leaves the household, a required training lapses, the home is placed on a corrective action or development plan, or an investigation results in a finding, the case manager reviews the credential at the next quarterly foster home evaluation.

4. **Loss of a credential.** When a home no longer meets the requirements, the Program Director suspends the credential, notifies the SSCC or DFPS caseworker for each child served in the home under that package, and works with the caregivers on a written plan to restore it.
5. The number of credentialed homes and the on-time completion of re-credentialing are reviewed through the Continuous Quality Improvement process.

T3C-11 Kinship Caregiver Support Services Add-On

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To set how Benevolent House engages kinship caregivers, helps them through verification, and provides ongoing support and enhanced technical assistance.

POLICY

Kinship caregivers step in for a child they know and love, often with little notice. Benevolent House adapts its services to the child, the caregiver, and the home, so that the placement is safe, verified, and able to last. Kinship support is delivered together with the child's primary Service Package and follows the Treatment Model.

PROCEDURE

A. Engagement and verification

1. Kinship Caregiver Home Support staff contact the caregiver promptly after the referral and meet with the caregiver in the home.
2. Staff explain each step of foster home verification and give the caregiver a written list of what is needed, who will help, and target dates.
3. Staff help the caregiver complete each step: background checks, the home study, health and fire inspections, pre-service training, and needed changes to the home. Staff help the caregiver find resources for items the home needs to meet Minimum Standards.
4. Training and meetings are scheduled around the caregiver's work and family, including evenings and weekends.
5. The date of referral and the date of verification are recorded for each kinship home.

B. Ongoing support and technical assistance

1. Kinship Caregiver Home Support staff are trained in the state and federal benefits a kinship caregiver may be eligible for while caring for a child in paid foster care, and in the timing and steps to apply. Staff help the caregiver complete and submit applications and other documentation, and record each application assisted.
2. Staff prepare a kinship support plan with the caregiver that lists the supports, training, and benefits the family needs, and review it at each monthly visit.
3. Staff coach the caregiver in using TST-FC within family relationships, including contact with the child's parents, grief and loss, and changes in family roles.
4. Staff are available to work with the caregiver on weekends and outside normal business hours.
5. Staff are available 24 hours a day, 7 days a week, to give an immediate response to a kinship caregiver. Each kinship caregiver receives the on-call number at the first visit.
6. Staff help the caregiver prepare for permanency, including adoption or Permanent Managing Conservatorship with Permanency Care Assistance, and provide Aftercare Services as described in Policy T3C-9.

C. Measures

Benevolent House tracks the time from referral to verification, placement stability, and the percent and timeliness of permanency exits to reunification, relative adoption, and relative Permanent Managing Conservatorship with Permanency Care Assistance, for all children living in a kinship foster family home. Results are reviewed by kinship home and by Kinship Caregiver Home Support staff member through the Continuous Quality Improvement process.

T3C-12 Paid Intermittent Alternate Care

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To support caregiver wellness and retention through a paid Intermittent Alternate Care program for children served under T3C.

POLICY

Intermittent Alternate Care, also called respite care, is planned alternative 24-hour care for a child that lasts more than 72 consecutive hours. For a child served under a T3C Service Package, Benevolent House pays the caregiver who provides Intermittent Alternate Care, and the child's own caregiver continues to be paid for the same days. For children served under T3C, this policy takes the place of the respite payment protocol in the Foster Parent Rights and Responsibilities agreement.

PROCEDURE

1. Up to 7 days of paid Intermittent Alternate Care may be used for each child in a fiscal year, September through August.
2. The caregiver requests Intermittent Alternate Care from the case manager, at least 10 days ahead whenever possible. The case manager approves the request and notifies the SSCC or DFPS caseworker.
3. Care is provided in a verified Benevolent House foster family home whose caregivers have completed TST-FC training. For a child in the Mental & Behavioral Health Support Services Package, the home is also verified to provide treatment services for emotional disorders.
4. Before the stay, the case manager gives the Intermittent Alternate Care caregiver the child's Managing Emotions Guide, safety and crisis plan, medication and medical information, and schedule.
5. For each approved day, Benevolent House pays the Intermittent Alternate Care caregiver the same daily foster family rate for the child's Service Package that the child's own caregiver receives, and the child's own caregiver continues to receive that rate. Any Add-On Service amount that passes through to the foster family continues to be paid to the child's own caregiver.
6. Both payments are made on the regular foster parent payment date and are recorded in the billing system by child, by home, and by date.
7. **Exceptions.** Both caregivers are not paid for the same day when the 7 days for that child have been used for the fiscal year; when the alternate care lasts 72 consecutive hours or less; or when the child is moved from the home because of an investigation, a safety plan, or a caregiver's request for discharge. In these cases the caregiver who provides the care is paid for the days of care provided.
8. Use of Intermittent Alternate Care by each home is reviewed through the Continuous Quality Improvement process as a measure of caregiver support.

T3C-13 Human Trafficking Prevention

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To make sure all staff and caregivers are trained to prevent and recognize human trafficking, and that each child receives prevention information in a planned way that fits the child's age.

POLICY

All staff and caregivers complete the DFPS Universal Human Trafficking Prevention Training. Every child, youth, and young adult served under T3C receives information on the prevention of human trafficking according to the method below.

PROCEDURE

A. Staff and caregivers

1. The training is delivered by a Benevolent House trainer who has completed the DFPS Universal Human Trafficking Prevention Train-the-Trainer course.
2. Staff complete the training during orientation and every 12 months. Caregivers complete it during pre-service training and every 12 months.
3. Completion is recorded on the training record with the date, trainer, and hours.

B. Children, youth, and young adults

Age	Method and content
Under age 5	The caregiver teaches body safety and safe adults through everyday routines. The case manager reviews prevention and supervision with the caregiver.
Ages 5 through 11	The case manager and caregiver talk with the child about body safety, safe and unsafe secrets, trusted adults, and safety online and with phones.
Ages 12 and older	The case manager talks with the youth about what trafficking is, how people are recruited in person and online, healthy and unhealthy relationships, the added risk when a youth leaves placement without permission, and how to get help, including the National Human Trafficking Hotline at 1-888-373-7888 or by text at 233733.

1. The information is provided as part of the child's initial Service Plan and at least once every 12 months.
2. The case manager records the date, the method used, and who took part in the Service Plan or the monthly visit note.
3. Any concern that a child has been or is being trafficked is reported immediately to the Texas Abuse Hotline and the SSCC or DFPS caseworker and handled as a serious incident.

Attachment A What Helps Me Feel Safe

A guide for children and youth at Benevolent House

Everybody has an alarm

Your body has an alarm that goes off when something feels scary or unsafe. When hard things have happened before, the alarm can go off fast, even when you are safe now. That is not your fault. It is your body trying to protect you.

What the alarm can feel like

- **Calm.** I feel okay. I can think, play, and learn.
- **Getting upset.** My body feels tight or jumpy. I might get loud, quiet, or want to leave.
- **Alarm is on.** It is hard to think or listen. I might yell, run, or freeze.
- **Coming back.** I am calming down. I might feel tired or sad.

What the adults here will do

- Keep you safe.
- Tell you ahead of time what is going to happen.
- Stay calm and stay close, or give you space if that is what helps.
- Wait until you are calm to talk about what happened.
- Help you fix things without shaming you.

Your plan

You and your caregiver will make a plan together. It is called a Managing Emotions Guide. It says what sets off your alarm, how people can tell you are getting upset, and what helps you. It is your plan, and you can change it.

You have a say

You can tell your caregiver or your case manager what is working and what is not. You can come to your Service Plan meetings and share what you want for yourself.

Attachment B Continued Stay Written Confirmation

Item	Entry
Child (initials and Person ID) and foster family home	
Service Package, and Kinship Add-On (yes or no)	
Date of Service Plan review	
Date of most recent CANS 3.0 Assessment	

Guideline	Yes	No
Services have been reviewed and adjusted based on the most recent CANS 3.0 Assessment and the Service Plan.		
The primary reason the child met the Admission Guidelines continues to require ongoing services, or has been replaced by other needs that fit this Service Package.		
Services continue to support the child's safety, well-being, and permanency in accordance with the child and family Service Plans.		
A less restrictive placement type is not appropriate to meet the child's needs.		
Mental & Behavioral Health only: the Basic package is not appropriate to meet the child's needs.		
Benevolent House and the foster family home hold the Credential for this Service Package.		
The child's goals and services align with the child's strengths, needs, and permanency plan.		

Confirmation. I have reviewed the child's goals and services. The child continues to meet the criteria for this Service Package and is benefitting from the Trauma Systems Therapy for Foster Care Treatment Model offered through the program.

Summary of the child's progress and basis for this confirmation

Role	Signature	Date
Program Director (or Executive Director, per T3C-S1)		
Treatment Director (Mental & Behavioral Health)		

Notification	Entry
Copy sent to SSCC or DFPS caseworker (name)	
Date sent (within 15 business days of review) and sent by	

Attachment C Aftercare Services Plan

Item	Entry
Child (initials and Person ID), and Service Package or Add-On Service	
Date of discharge or permanency, and discharged to (relationship and placement type)	
Aftercare Case Manager or Aftercare Kinship Support staff: name, phone, email	
STAR Health Service Coordinator, when assigned: name and phone	
SSCC or DFPS caseworker: name and phone	

Continued service or benefit	Provider and contact	Referral date and first appointment

Provided at discharge	Yes	Not applicable (reason)
Education Portfolio		
Managing Emotions Guide and strategies that have worked		
Medication and medical information		
Kinship support group information (Kinship Add-On)		

Follow-up schedule. Mental & Behavioral Health: twice a month for 6 consecutive months. Kinship Add-On: weekly for the first month, then twice a month, for 6 months.

Month	Contact dates and times	Method	Completed by
1 and 2			
3 and 4			
5 and 6			

Role	Signature	Date
Youth, when appropriate		
Caregiver or family receiving the child		
Aftercare staff member		

Attachment D Foster Family Home T3C Credential Record

Item	Entry
Foster family home	
Caregivers	
Type of review (initial credential or annual re-credential)	
Credential requested (Basic; Mental & Behavioral Health; Kinship Add-On)	
Date of home visit for this review	

Requirement	Date met	Met	Not met
Verification current and in good standing with Minimum Standards			
TST-FC eight-hour training and Final Knowledge Check, and quarterly TST-FC sessions current, each caregiver			
Universal Human Trafficking Prevention Training, each caregiver			
T3C Policy and Procedure Training, each caregiver			
Pre-service or annual training, background checks, and home documents current			
Quarterly foster home evaluation shows use of the Treatment Model			
Mental & Behavioral Health: verified for treatment services for emotional disorders			
Mental & Behavioral Health: emergency behavior intervention, suicide prevention, and psychotropic medication training			
Mental & Behavioral Health: able to take part in therapy, attend meetings, and respond immediately			
Kinship Add-On: relative or fictive kin of the child; kinship support plan in place			

Items remaining and plan to complete them, when any requirement is not met

Decision	Credential	Signature	Date
Case manager recommendation			
Case Manager Supervisor review			
Program Director approval			

Next re-credentialing due (12 months from approval): _____